

PHILHEALTH CIRCULARNo. 2016 - 0009**TO : ALL ACCREDITED HEALTHCARE PROVIDERS,
PHILHEALTH MEMBERS, PHILHEALTH REGIONAL
OFFICES AND ALL OTHERS CONCERNED****SUBJECT : Quality Standards on the Diagnosis and Management of
Dengue (Adult and Children) as a Reference of the
Corporation****I. RATIONALE**

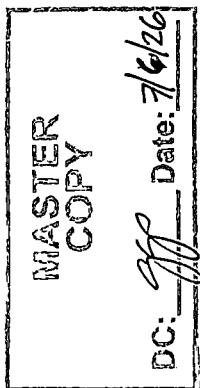
The Universal Health Care Act of 2019 (R.A. No. 11223) mandates that the Corporation shall support the implementation of clinical standards of care established by the Department of Health (DOH) in collaboration with duly recognized specialty societies, ensuring that health care services are evidence-based and patient-centered. Furthermore, pursuant to the revised Implementing Rules and Regulations (IRR) of the National Health Insurance Act of 2013 (R.A. No. 7875 as amended by R.A. Nos. 9241 and 10606), PhilHealth applies quality assurance measures to maintain and improve the quality of care services.

Dengue remains a significant and endemic public health concern in the Philippines, with recurrent outbreaks especially during the wet season. Early detection, accurate diagnosis, and proper management are vital preventing disease progression and reducing morbidity and mortality.

The standards outlined in these statements were developed through consultations with the Philippine College of Physicians (PCP), and the Philippine Academy of Family Physicians (PAFP), and were further refined based on recommendations from the Philippine Pediatric Society (PPS). Furthermore, the patient's perspective was incorporated, with support from the Philippine Alliance of Patient Organizations (PAPO).

II. OBJECTIVES

This PhilHealth Circular aims to uphold the minimum standards of care in the diagnosis, prevention, and management of Dengue. It is subject to periodic review and revision based on evolving evidence and healthcare practices.



III. SCOPE

This PhilHealth Circular outlines the minimum expected care pertaining to the diagnosis and management of Dengue. Furthermore, this evidence-based policy shall apply to all patients hospitalized due to dengue.

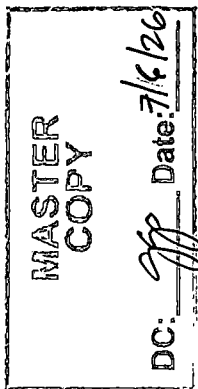
IV. DEFINITION OF TERMS

A. Dengue – also known as breakbone fever- is a systemic viral infection with diverse clinical manifestations and a dynamic disease course. It is transmitted to humans through the bite of *Aedes aegypti* mosquitoes, typically causing a severe flu-like illness that can progress to life-threatening complications known as severe dengue.¹

V. POLICY STATEMENTS

A. Assessment of Dengue

1. Patients diagnosed with dengue fever are evaluated through a comprehensive medical history, detailed physical examination, and laboratory investigations.
2. Dengue infection typically presents with intermittent fever lasting 2-7 days, with temperature ranging 38 °C and above, accompanied by two or more of the following clinical manifestations:
 - a. Headache
 - b. General body malaise
 - c. Myalgia (muscle pain)
 - d. Arthralgia (joint pain)
 - e. Retro-orbital pain
 - f. Generalized maculopapular rash
 - g. Vomiting
 - h. Poor Appetite
 - i. Hemorrhagic manifestations
 - j. Thrombocytopenia ($<150 \times 10^9/L$) and Leucopenia ($<5 \times 10^9/L$)



B. Laboratory and Diagnostic Tests

The following are various laboratory and diagnostic tests to support the diagnosis of dengue fever:

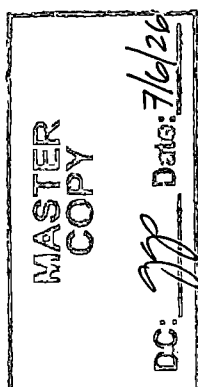
1. Complete Blood Count (CBC) with platelet count is an important measure during the acute stage of dengue infection.

¹World Health Organization. (2025, August 21). Dengue and severe dengue. <https://www.who.int/news-room/factsheets/detail/dengue-and-severe-dengue>

2. Non-Structural Protein-1 (NS1) antigen can be detectable up to day seven (7) to day nine (9) of illness. However, a negative NS1 result does not rule out dengue infection.
3. The use of dengue IgM and IgG with or without NS1 antigen RDT is recommended for patients presenting more than 5 days from symptom onset, especially in resource-limited areas (i.e., GIDA), where these are the only available and accessible tests.
4. Dengue RT-PCR may be requested if available to support the diagnosis.
5. Imaging techniques such as chest radiography may support the identification of plasma leakage in patients with severe dengue.
6. Additional laboratory and diagnostic tests, as indicated and as assessed by the healthcare provider, may be done:
 - a. Blood glucose
 - b. Serum electrolytes (sodium, potassium, and calcium)
 - c. Liver Enzyme Tests (AST/SGOT, ALT/SGPT)
 - d. Renal function tests (urea, creatinine)
 - e. Prothrombin Time (PT), Partial Thromboplastin Time (PTT) or severe dengue

C. Criteria for Hospital Admission

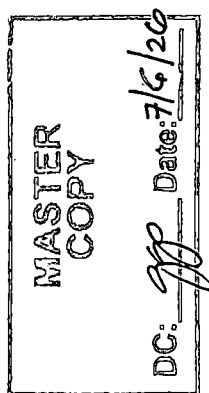
1. Patients with dengue who do not exhibit warning signs (Annex A: Dengue Case Classification) may be observed at home. However, if there is no clinical improvement and any of the warning signs are present below, the patient must be taken to the hospital for timely and appropriate medical attention:
 - a. Condition worsens as the fever subsides
 - b. Altered level of unconsciousness (lethargy, confusion, or decreased responsiveness)
 - c. Inability to take in oral intake
 - d. Difficulty of breathing
 - e. Severe abdominal pain
 - f. Persistent vomiting
 - g. Irritability/restlessness
 - h. Bleeding (gums, nose, gastrointestinal)
 - i. Excessive menstrual and/or intermenstrual bleeding
 - j. Black stool or coffee-ground vomiting
 - k. No urine output for more than six (6) hours
 - l. Tachycardia
 - m. Tender hepatomegaly >2cm below the costal margin
 - n. Leucopenia ($< 5 \times 10^9/L$)
 - o. Increasing hematocrit
 - p. Rapidly decreasing platelet count
 - q. Clinical fluid accumulation



2. Patients with coexisting conditions, including but not limited to pregnancy, obesity, infants, the elderly, and individuals with comorbidities (but not limited to diabetes, renal diseases, respiratory diseases), as well as those living alone or in remote areas without immediate access to medical facilities, must be admitted to the hospital to receive appropriate medical care and avoid the critical period.
3. All patients with severe dengue must receive emergency treatment at a healthcare facility to ensure prompt, appropriate medical care.

D. Treatment and Management

1. To prevent and reduce morbidity and mortality from dengue, the disease must be identified immediately, which will enable healthcare services to administer and manage appropriate medical care.
2. Patients who do not require hospitalization or can be managed as outpatients must be advised of the following medical care:
 - a. Advise adequate bed rest.
 - b. Encourage and inform about the importance of adequate oral fluid intake or oral rehydration solution (ORS), and avoid sports drinks.
 - c. Paracetamol may be given for fever in both adults and children in a tablet or syrup, depending on age and weight, along with tepid sponging of the forehead, extremities, and axillary areas as supportive care.
 - d. Use of non-steroidal anti-inflammatory drugs (NSAIDs) such as ibuprofen, celecoxib, aspirin, and corticosteroids (dexamethasone) is not recommended.
 - e. Patients must be advised of the subsequent follow-up visit for close monitoring to prevent progression from mild to severe dengue.
 - f. Daily monitoring of complete blood count to monitor the elevation of hematocrit and a decrease in platelet count.
3. Dengue patients who exhibit warning signs and belong to high-risk groups require close monitoring and appropriate medical care.
 - a. The patient should be admitted to the hospital for appropriate clinical care.
 - b. Complete Blood Count (CBC) with platelet count must be obtained prior to initiating fluid management. If plasma leakage is detected, start intravenous fluids using an isotonic crystalloid solution. For children, computation should be based on the lower value between actual or ideal body weight.

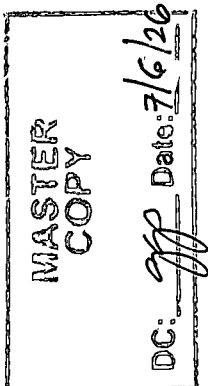


- c. Vital signs, perfusion parameters (capillary refill time, pulse pressure, pulse character, both central and peripheral pulses), and urine output should be closely monitored during the critical phase of dengue.
4. Patients with severe dengue require immediate emergency care and urgent medical treatment to prevent complications and reduce mortality.
 - a. Clinical parameters, including urine output, pulse rate, and heart rate, must be regularly assessed and closely monitored to prevent fluid overload during dengue management.
 - b. Initiate isotonic crystalloid (e.g., Ringer's lactate) solution or colloids, if necessary, using the lower of the actual and ideal body weights for dosage calculation.
 - c. Reassessment of the complete blood count, including the platelet count, and/or serum electrolytes should be performed as clinically indicated.
 - d. Vital signs, capillary refill time, pulse pressure, pulse character, central and peripheral pulses, and urine output shall be monitored every 30 minutes to 1 hour. During shock or while undergoing fluid resuscitation, all parameters, including urine output, should be monitored every 5 minutes.
 - e. Hematocrit levels, blood glucose, urine output, and other organ functions should be continuously monitored until the patient shows clear clinical improvement and is no longer in a hemodynamically unstable phase.
 - f. Blood transfusion may be performed when there is evidence of significant bleeding and/or failure to respond to fluid resuscitation, as indicated by a decrease in hematocrit to $>40\%$ in children and adult females or $>45\%$ in adult males.
5. The use of herbal medicine and acid suppressants for dengue infection is not recommended due to limited available evidence.

E. Hospital Discharge

Patients may be discharged when all of the following criteria are met:

1. The patient has been afebrile for at least 48 hours without taking paracetamol or antipyretics.
2. The patient demonstrates improved appetite and overall well-being, no longer requires intravenous fluid support, and is hemodynamically stable with adequate urine output.



3. Patients may be considered for discharge upon demonstration of stable vital signs and improving laboratory parameters, including a platelet count $>50,000/\text{mm}^3$, as determined appropriate by the healthcare team.

F. Monitoring and Evaluation

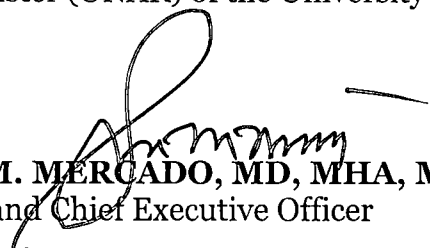
1. The healthcare provider shall be bound by the provisions of the Performance Commitment and subject to the rules on monitoring and evaluation of performance as provided in PhilHealth Circular No. 2018-0019 Health Care Provider Performance Assessment System (HCPPAS) Revision 2.
2. Standards of care issued by authorized agencies/organizations shall be regularly monitored. As deemed necessary, a revision of the policy statements shall be made.

VI. PENALTY CLAUSE

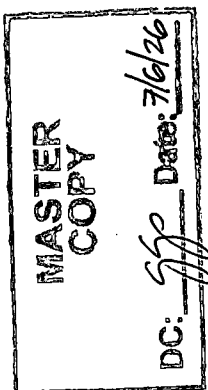
Any violation of this PhilHealth Circular shall be dealt with and penalized in accordance with pertinent provisions of R.A. No. 11223 and R.A. No. 10606, and their respective Implementing Rules and Regulations.

VII. DATE OF EFFECTIVITY

This PhilHealth Circular shall take effect fifteen (15) days after its publication in the Official Gazette or in any newspaper of general circulation. Three (3) certified copies shall be filed with the Office of the National Administrative Register (ONAR) of the University of the Philippines Law Center.


EDWIN M. MERCADO, MD, MHA, MMSc
President and Chief Executive Officer

Date signed: 06/29/2026



Quality Standards on the Diagnosis and Management of Dengue (Adult and Children) as a Reference of the Corporation

Annex A: Dengue Case Classification

Dengue without Warning Signs	Dengue with Warning Signs	Severe Dengue
Probable dengue Lives in or travels to a dengue-endemic area and has fever with atleast two of the following: • Headache • Body malaise • Myalgia • Arthralgia • Retro-orbital pain • Anorexia • Nausea • Vomiting • Diarrhea • Flushed skin • Torniquet test positive • Rash (petechial, Hermann's sign) AND Laboratory test • Complete Blood Count (CBC) with platelet count; and • Dengue NS1 antigen test or Dengue IgM and IgG Laboratory confirmed dengue fever: (if available) • Viral culture isolation • PCR	Lives in or travels to a dengue-endemic area and has fever lasting for 2-7 days, plus any one of the following: • Abdominal pain or tenderness • Persistent vomiting • Clinical signs of fluid accumulation • Mucosal bleeding • Lethargy, restlessness • Liver enlargement >2cm below the costal margin Laboratory parameters: • Increase in hematocrit and /or decreasing platelet count Laboratory confirmed dengue fever: (if available) • Viral culture isolation • PCR	Lives in or travels to a dengue-endemic area and has fever of 2-7 days and any of the clinical manifestations for dengue with or without warning signs, plus any of the following: Severe plasma leakage, leading to: • Shock • Fluid accumulation with respiratory distress Severe bleeding • Profuse bleeding from the nose and/or gums • Prolonged menstrual bleeding in between periods • Gastrointestinal bleeding • Blood in urine • Blood in stool Severe organ impairment • Liver: AST or ALT ≥ 1000 • Seizures, impaired consciousness • myocarditis • renal failure Laboratory test: • Complete Blood Count (CBC) with platelet count; and • Dengue NS1 antigen test or Dengue IgM and IgG test

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